

The placenta will be examined to ensure that it appears normal and isn't blocking the baby's exit route (the cervix). In early pregnancy the placenta may be [low-lying](#). This is more common in subsequent pregnancies. In most cases, the placenta moves up and out of the way by the third trimester as the uterus grows.

If your placenta is low lying, your doctor will arrange a follow-up scan later in pregnancy to check that the placenta has moved up out of the way. If a later scan shows that it has failed to move up, a condition known as placenta previa (see [Bleeding in late pregnancy](#)), you will be monitored until the birth. The amniotic fluid is assessed. Too little can indicate a problem with fetal growth or the baby's renal tract; it may also be due to early membrane rupture. Too much fluid can be normal but is also seen in diabetic pregnancies and, very rarely, in esophageal atresia, where the baby cannot swallow causing a buildup of amniotic fluid.

Uterine artery Dopplers

If you are at increased risk of developing preeclampsia, you may be offered a Doppler scan to measure the blood flow in the uterine arteries. If there is a resistance to the flow, you may be at increased risk of preeclampsia or growth problems in the baby and will be offered extra monitoring and scans.

Detecting markers

Ultrasound at this stage of pregnancy is not a reliable way to detect Down syndrome—only about 60 percent of babies will have markers or signs of Down syndrome. Some of these markers, for example, a bright spot in the baby's heart, extra fluid in the kidneys, and short arms or legs, can occur even when the baby is not affected by Down syndrome. Some specific abnormalities are linked with a far higher risk of Down or other chromosomal abnormalities. These include certain kinds of heart defects and other major malformations, such as abnormalities in the bowel.

An ultrasound scan can detect almost all babies with [Edwards and Patau syndromes](#) since these conditions are associated with very significant abnormalities. However, a diagnostic test will still be required.

What happens next If soft markers are detected, a [diagnostic test](#) may be suggested, especially if you already have an increased risk of Down syndrome. However, an earlier all-clear from a diagnostic test means that you can be confident your baby's chromosomes are normal. If an obvious abnormality is seen, and you haven't had a diagnostic test, this will probably be strongly advised. Whatever your decision, any associated abnormalities will be monitored.

Boy or girl?

Unless your baby is curled up or the legs are closed tight, the sonographer will have a good idea as to whether you're having a boy or a girl. If you don't want to know the sex,